

Our Ref: (T) SW/gd/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division C

Southampton Children's Hospital Paediatric Medicine

D Level, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS], Southampton, [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Problems:

Four month history of frequent loose stools, intercurrent abdominal pain and unexpectedly raised inflammatory markers.

Raised Calprotectin at 121

Bloods taken by the GP on [DATE] secondary to abdominal pain:

Haemoglobin of 184, white count cells of 15.2, neutrophils 11.9, platelets of 350

CRP of 192, ferritin of 104

normal calcium, liver function, renal function, amylase of 49,

TTG Normal

Bloods repeated on [DATE]

CRP of 130, ESR of 95.

Chest X-ray reported as completely normal with no signs of infection.

Stool pathogen panel negative for Giardia, cryptosporidium, E. coli, campylobacter, Shigella Salmonella, C. Diff toxin negative, norovirus not detected.

Abdominal ultrasound on the [DATE] showed normal liver, normal gallbladder, normal pancreas, normal, right kidney and left kidney showed no abnormalities, no adenopathy was seen. [NAME] volume of free fluid in both iliac fossa. The appendix was identified in a retrocecal location. [NAME] was not seen, the proximal segment did not appear to show localised inflammatory change. [NAME] colonic or small bowel thickening identified on high-resolution settings.

Faecal calprotectin [DATE]

Raised at 121

Bloods 6th December:

Full blood count showed a haemoglobin of 112, white cells 6.1, neutrophils 2.1, platelets of 350, ESR has decreased to 25, although is still above the normal range, CRP now 2,

Age: 5 years and 10 months

Height 111.7cm showing sustained linear growth over the last three years on the 25th centile, growth arrest noted

Weight 21.5kg currently on the 75th centile having previously been up to the 91st centile

BMI plotting between the 75th and 91st centile

Mid parental height between the 9th and the 91st centile.

I reviewed [NAME] on Paediatric Assessment Unit a week after her discharge following her admission secondary to increased abdominal pain and inflammatory markers.

To summarise, [NAME] was known to myself previously with a history of chronic constipation with diarrhoea and was treated with Movicol and senna for that. Since the end of September, [NAME] has had intermittent fever, coughs and coryza with abdominal pain and increasing frequency of the stools, stooling overnight and during the day. [NAME] is otherwise thriving and gaining weight. [NAME] has had intermittent vomiting associated with abdominal pain and stooling overnight as well as during the day. [NAME] does stool after meals rather than during meals and has had mouth ulcers since September. [NAME] weight has been stable. [NAME] does suffer from abdominal bloating. [NAME] was started on amoxicillin and ceftriaxone on 27th November and was treated for five full days. Over that time her stools d

improve and on review today, [NAME] continued to have frequent loose stools, four a days associated with meals although not overnight.

On review today, [NAME] had her bloods repeated and you can see those are appended above with normalisation of her ESR.

On examination today, [NAME] was not clubbed, [NAME] had a soft non-tender tummy with no abdominal tenderness and certainly no faeces palpable.

Impression:

[NAME] is a young lady who has had a four-month history of mouth ulcers, increasing abdominal pain, frequent loose stools with unexpectedly and unexplained high inflammatory markers, that seemed to have improved entirely on a course of IV antibiotics.

Plan:

I discussed [NAME] with the paediatric Gastroenterologists at the time of her admission and following the Calprotectin result will refer her formally for an opinion.

I have continued to follow her up in my outpatient clinic and have safety netted mum that should her school attendance at school deteriorate or her abdominal symptoms increase, then I would like to be contacted on the above telephone number to be informed prior to my next review which is currently booked for [DATE] at 13:30.

Should [NAME] have deterioration of her symptoms over the next week, [NAME] has the open access to the Paediatric Assessment Unit and following that time.

I do not feel that a further repeat of [NAME]'s bloods is currently indicated providing [NAME] remains clinically stable.

Thank you very much for your help with this young lady.

Yours sincerely

[NAME]

[NAME]

[DICTATION-META]

[ADDRESS]

Copies to:

[ADDRESS]

[NAME]

co/ Parents

Parent or Guardian of Miss [NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]